EVIDENCE FILE / SOURCE 1

What the invoice leaves out

Use these sources to shape a local renewal review. Your costs, patient choices and operational results come from your own records.

Paying for clinical AI: The bill is not the cost

Vishva Natarajan, Caleb Stewart, Jennifer Hong and Saeed Hassanpour. PLOS Digital Health, November 2, 2026. Opinion.

Open publisher full text / 10.1371/journal.pdig.0001814

What the authors examined

The opinion describes how usage-based fees, fixed licenses and local hosting put costs in different places. It calls attention to costs the vendor may never invoice, including engineering and oversight.

How to use it in a renewal review

Ask procurement and IT for contract schedules, usage rules, invoices and implementation ledgers. Separate one-time and recurring spend on the same renewal horizon. Add local staffing and support observations to see who bears the work.

What it cannot establish

The opinion does not measure comparative hospital costs, savings or return on investment. Treat its examples as questions to test. Use local contracts and operating records to estimate spend, and use payroll, ledger or verified collected contribution to substantiate a cash benefit.

What to bring

Price schedule, invoices and any overage rules. Add the local implementation ledger, staffing observations, ongoing oversight costs and the worksheet's missing data owners.

EVIDENCE FILE / SOURCE 2

Why patient choice matters

Patient choice affects how a clinic staffs both documentation paths. Test what happens in your own clinic.

Patient expectations for artificial intelligence transparency and agency

Guy Stein, Abinav R. Udaiyar, David Bouslov, Howard M. Ross and Charles E. Binkley. PLOS Digital Health, November 4, 2026. Qualitative study.

Open publisher full text / 10.1371/journal.pdig.0001816

What the researchers heard

The researchers interviewed 20 Patient and Family Advisory Council members from nine sites in one health network. Participants wanted to know when AI was involved and valued clinicians retaining decision authority. The authors propose disclosure, assent and consent approaches scaled to the stakes.

How to use it in a renewal review

Have clinical operations, privacy and patient experience define who explains recording, how choice is recorded, and the path after a decline or withdrawal. Test conventional documentation during a normally staffed session, including interpreter-supported visits and tool failures.

What it cannot establish

The small advisory sample was predominantly female, White and highly educated. It offers no local decline rate, representative estimate or causal performance result. The authors propose a communication framework. Determine applicable requirements with your privacy and legal teams.

What to bring

Use a locally approved choice log, encounter roster and signed-note data. Keep declines and failures separate. Use all eligible completed encounters when comparing the two workflows.